



THE HOSPICE LANDSCAPE

A NATIONAL REVIEW OF HOSPICE OWNERSHIP MODELS OVER TIME

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Executive Summary

The Hospice Landscape: A National Review of Hospice Ownership Models Over Time

BACKGROUND

The composition of the U.S. hospice system has shifted substantially since 2018, driven by rapid expansion of for-profit ownership and growing private equity investment. This report maps that transformation, illustrating how ownership models have shifted from 2018 to 2025, including the increasing involvement of private equity (PE) investments.

The analysis provides an overview of the current hospice landscape, including where hospices are most concentrated, what the ownership models look like, and if they are receiving PE funding. Beyond a current landscape assessment, this report shows how that landscape differs from the 2018 baseline.

KEY FINDINGS

This analysis fills a critical gap in current, publicly available data on the hospice market, examining the landscape in three main ways—the national footprint, shifts in ownership models, and the growing role of private equity.

THE NATIONAL LANDSCAPE

+46%

growth in 8 years

The U.S. hospice market expanded substantially between 2018 and 2025, growing from 4,744 to 6,921 Medicare-certified hospices, a 45.9% increase in just eight years.

Growth has accelerated sharply in recent years, but that expansion has not been evenly distributed across the country. Nevada, California and Arizona led growth with increases of 251%, 128%, and 83%, respectively, while 12 states saw their hospice markets contract.

THE SHIFT TO FOR-PROFIT OWNERSHIP

>50%

*growth in for-profit
hospices*

For-profit hospices drove a large portion of the market's expansion, growing by 51.1% to reach 4,747 hospices by 2025. Over the same period, nonprofit/government-operated hospices declined by 22.2%.

The hospice landscape is now primarily for-profit, and the gap is widening. This continued contraction of the nonprofit and government segment may have implications for the diversity of providers whom patients can choose from when looking for care.

THE ROLE OF PRIVATE EQUITY IN HOSPICE

>38%

*of for-profit hospices
with documented
ownership changes
were PE-associated*

Among for-profit hospices with documented change of ownership (CHOW) transactions, more than 1 in 3 (38.7%) were associated with private equity funding.¹

This figure is likely a conservative estimate of the true role that PE plays in the hospice market. Under current regulations, Medicare enrollment records do not require PE disclosure, and many ownership transfers occur through stock sales that bypass formal reporting requirements, full attribution is not possible with available public data. However, what is visible is striking: PE-backed ownership is a substantial and likely undercounted feature of the for-profit hospice market.

WHAT THIS MEANS

Taken together, these findings point to increasing consolidation around larger, for-profit, and in many cases PE-backed hospices, as the nonprofit and government segment continues to contract. Linking these ownership shifts to patient-level data on care utilization, quality, outcomes, and costs is the critical next step to know how the rapidly changing hospice landscape is affecting the patients and families at the center of it.

¹ This analysis combines publicly available data from CMS and proprietary data from [PitchBook](#), a Morningstar company. Healthsperien conducted this analysis using accepted best practices. The cited data has not been reviewed by PitchBook analysts and may be inconsistent with PitchBook methodology. Please see full report and methodology for more details.

Introduction

Background on the U.S. Hospice System

Hospice care is a [Medicare-covered benefit](#) providing palliative and end-of-life care to individuals in the final stages of a terminal prognosis. Medicare is the primary payer for hospice services in the United States, making the landscape of Medicare-certified hospice providers an important priority to understand where care is being provided and by whom.

The U.S. hospice system has undergone significant structural change over the past decade, driven by shifts in payment policy, patterns of market entry and exit, and the increasing involvement of private equity (PE) investments. Despite the significant changes, there are limited, up-to-date analyses of the hospice market landscape. To address this gap, this report provides a systematic account of how the hospice landscape has evolved between 2018 and 2025, with a particular focus on the growth of for-profit ownership and the expanding footprint of PE investments.

Approach

This analysis examines the Medicare-certified hospice landscape at two points in time, 2018 and 2025, using CMS Care Compare data. The CMS Care Compare data provides information on hospices' name, location, and self-reported ownership type for all certified providers. To construct each point in time analysis, hospices were identified by unique CMS Certification Number (CCN); 2018 files were merged and deduplicated across all four quarterly releases to address reporting inconsistencies, while the final 2025 quarterly file was used to reflect the most current landscape.

An individual hospice is defined by a single CMS Certification Number, or CCN.

Each CCN corresponds to a single licensed hospice location, therefore multi-site hospice organizations that operate under a single parent company are counted as multiple hospices.

The two files were then linked by CCN to classify each hospice as operating in 2018 only, 2025 only, or operating in both years. Ownership type reflects self-reported data submitted through Provider Enrollment, Chain and Ownership System (PECOS) and is

categorized as for-profit, nonprofit/government, or other, following the classification methodology developed by the Assistant Secretary for Planning and Evaluation (ASPE).

After creating the hospice landscape in each year, we examined ownership changes across years. We combined CMS Care Compare data with CMS Hospice Change of Ownership (CHOW) files, which record formal ownership transactions—including effective date and buyer and seller CCNs.

Among hospices that had a CHOW transaction, we reviewed data from [PitchBook](#), a Morningstar company, to quantify the role of PE in the hospice landscape. We analyzed PE-related deals associated with hospices with a filed CHOW transaction and flagged as for-profit in the Care Compare data. This approach enabled us to provide further detail on whether those for-profit hospices could be classified as *for-profit with PE funding* or *for-profit without PE funding*.

For a more detailed methodology, see the appendix.

National Landscape of Hospices

Overall Hospice Landscape

The national hospice landscape expanded substantially over the study period. In 2018, CMS Care Compare data included 4,744 certified hospices; by 2025 the market grew to 6,921 hospices. This expansion reflects a 45.9% growth in the market over the eight-year period. While the hospice market has experienced consistent growth, the pace at which the market is growing appears to have accelerated in recent years. According to one [ASPE report](#), there were fewer than 5,900 hospices in 2022, suggesting that there was an increase of more than 1,000 new hospices between 2022 and 2025 alone.²

4,744

Hospices in 2018

6,921

Hospices in 2025

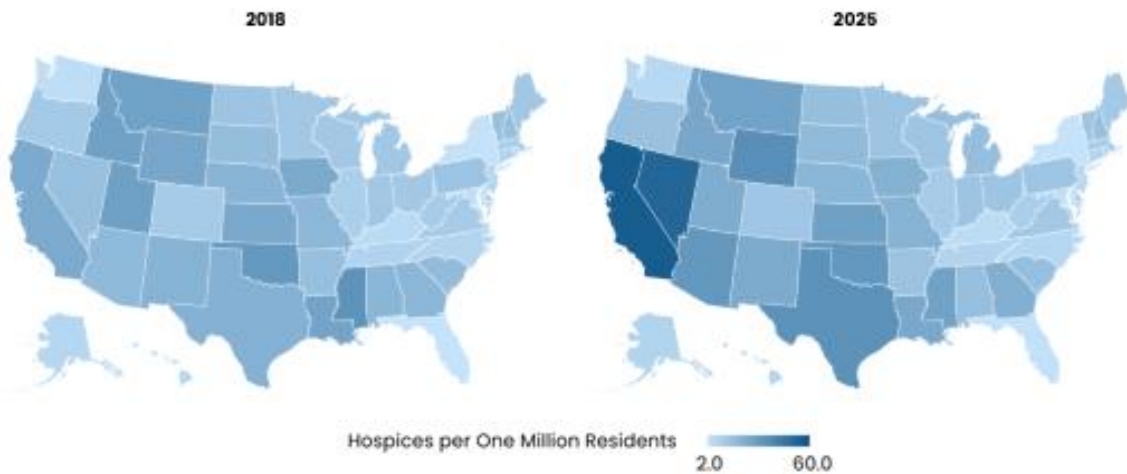
Geographic Distribution

The hospice landscape was not uniformly spread across states or regions. We examined how the national landscape was distributed across states to identify any areas that may have potential access limitations. To better understand where finding a hospice provider may be a challenge for patients, we analyzed the count of hospices per one million residents, by state (Figure 1).³ In 2018, Mississippi had the greatest number of hospices per one million residents (34.2), but substantial growth in California and Nevada's hospice markets resulted in more than 50 hospices per one million residents (55.9 and 52.4, respectively) by 2025.

² Hospice estimates may vary between this study and the cited ASPE report due to differences in approaches. Our 2018 count (4,744) is approximately 3% higher than the ASPE estimate, reflecting our deduplication across all four 2018 quarterly Care Compare files rather than an average of all files. See our methodology appendix for our detailed approach.

³ Hospices per one million residents were calculated using annual census estimates for both [2018](#) and [2025](#).

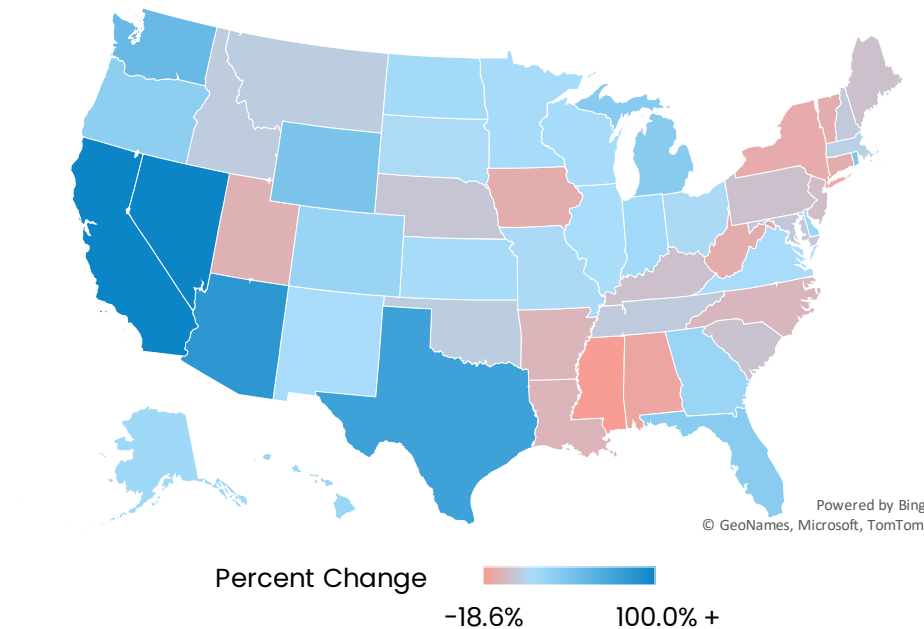
Figure 1. Count of Hospices per One Million Residents, by State, 2018 and 2025



The density of hospices per one million residents reflects how variation in hospice counts shifted at the state level from 2018 to 2025. Some states experienced growth in their hospice markets, while others contracted over the study period (Figure 2). While the majority of states did experience growth, there were 12 states that had reductions in their hospice market and three states that had a net change of zero between 2018 and 2025.

The range, from growth to reductions, in state hospice markets fluctuated substantially by state, from a 251.0% growth (Nevada) to a -18.6% reduction (Mississippi). Nevada, California, and Arizona experienced the greatest growth in their hospice market from 2018 to 2025, with 251.0%, 127.8%, and 82.8% growth, respectively. Conversely, Mississippi, Alabama, and New York had the three greatest reductions in their hospice markets.

Figure 2. Percent Change of Hospice Markets, by State, 2018 to 2025



Hospice Ownership Models

The overall growth in the hospice landscape was not distributed evenly across ownership models. Using the ownership classifications (tax status) as reported in CMS Care Compare data, we examined how the composition of the hospice landscape shifted from 2018 to 2025.⁴ This report quantifies how the landscape is increasingly becoming for-profit, as the share of nonprofit/government hospices continues to decrease.

The hospice landscape overall grew by 45.9% between 2018 and 2025, from 4,744 to 6,921 hospices, but the change in the landscape by ownership type tells two different stories about that growth. For-profit hospices expanded by more than 50% over that period, adding more than 1,600 hospices to reach 4,747 by 2025. The size of the

⁴ Due to the limitations of the Care Compare data, ownership models are limited to (1) for-profit, (2) nonprofit/government, and (3) other (see methodology for detailed approach).

nonprofit and government-operated hospice market, meanwhile, contracted, falling by more than 250 hospices, a decline of 22% (Table 1).

Among hospices operating in 2025, there were 787 hospices that did not have any ownership information associated with their CCN and are therefore not able to be categorized across the three ownership models.⁵ These hospices represent 11.4% of the 2025 hospice landscape. The absence of ownership data for this group introduces uncertainty into the ownership distribution findings, as they, most likely, span all ownership types—meaning each category is modestly undercounted than the available data reflect.

Table 1. Count of Hospices by Ownership Models, 2018 and 2025

	2018	2025
Total	4,744	6,921
For-Profit	3,142	4,747
Nonprofit / Government	1,169	910
Other	433	477
<i>Missing</i>	–	<i>787</i>

Changes in Ownership

To examine changes in ownership over the study period, we analyzed CMS Change of Ownership (CHOW) transaction data. CMS requires Medicare providers, including hospices, to file public CHOW transactions to document formal changes in hospice

⁵ After identifying 787 hospices with a blank ownership data field in Care Compare's November 2025 data file, we tracked each CCN through all available files of Care Compare data back to February 2018. All 787 hospices had incomplete data for all reports in which that CCN was found. Hospices without ownership information in the Care Compare data are included in the overall hospice landscape, but excluded from subsequent analyses by ownership type.

operational control.⁶ These transactions are publicly available and maintained by CMS and include information on both the buyer and seller.

We identified 390 CHOW transactions associated with the hospice landscape from between 2018 and 2025. The majority (351) of these transactions were associated with hospices that were operating in both 2018 and 2025, with the remaining 39 being associated with hospices that were new facilities in 2025.⁷

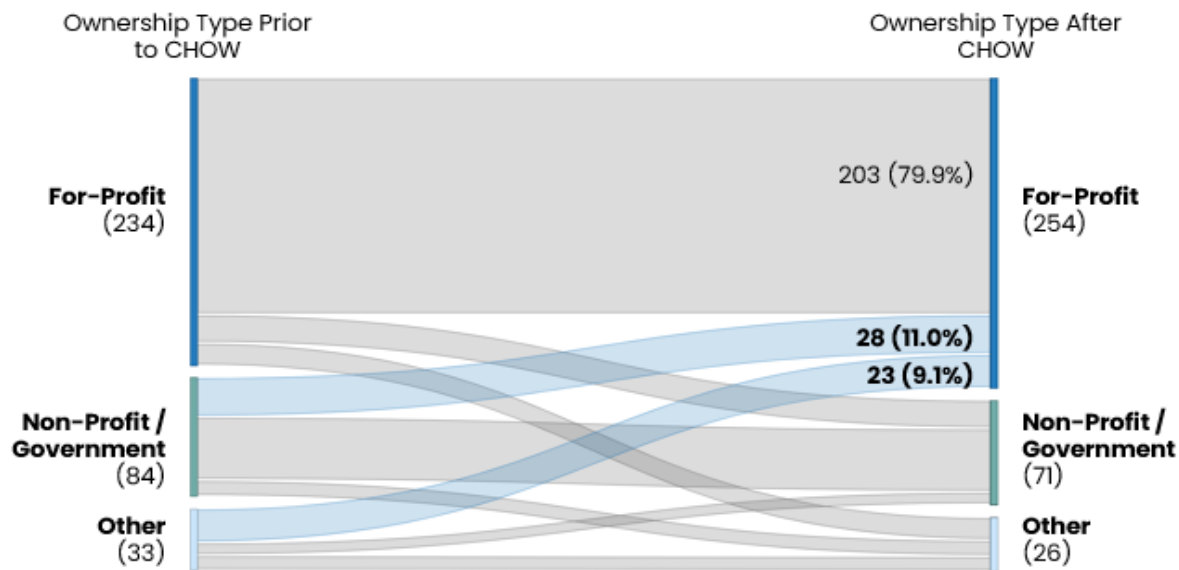
Due to the CMS definition of a change of ownership that requires hospices to file a formal CHOW transaction, these transactions only record a subset of ownership activity. Changes that effectively change who owns or controls a hospice, but do not require a CHOW filing (e.g., stock sales), are not captured in this analysis. Despite these limitations, however, CHOW transactions are still the best source of publicly available information to examine changes in ownership.

Among the 351 CHOW transactions that were associated with hospices operating in both 2018 and 2025, less than 1 in 3 (26.5%; 93 hospices) changed ownership type (tax status) whereas the majority of hospices had no change. Among those 93 hospices that changed ownership type following the CHOW transaction, more than half (54.8%) became for-profit (Figure 3).

⁶ CHOW transactions are statutorily required when there is a change of partnership, unincorporated sole proprietorship, corporation, or leasing (as outlined in 42 CFR 489.18).

⁷ We identified a total of 390 CHOW transactions that corresponded to our hospice landscape. The remaining 39 were attributed to hospices that were not listed as a CMS-certified hospice in the 2018 Care Compare facility and therefore flagged as a “new facility” rather than being included in the cohort of hospices operating in both 2018 and 2025.

Figure 3. Change in Ownership Transactions Among Hospices Operating in 2018 and 2025



Note: Figure reflects hospices that were operating in both 2018 and 2025 that experienced a CHOW (N=351).

Deep Dive: Private Equity in Hospice Care

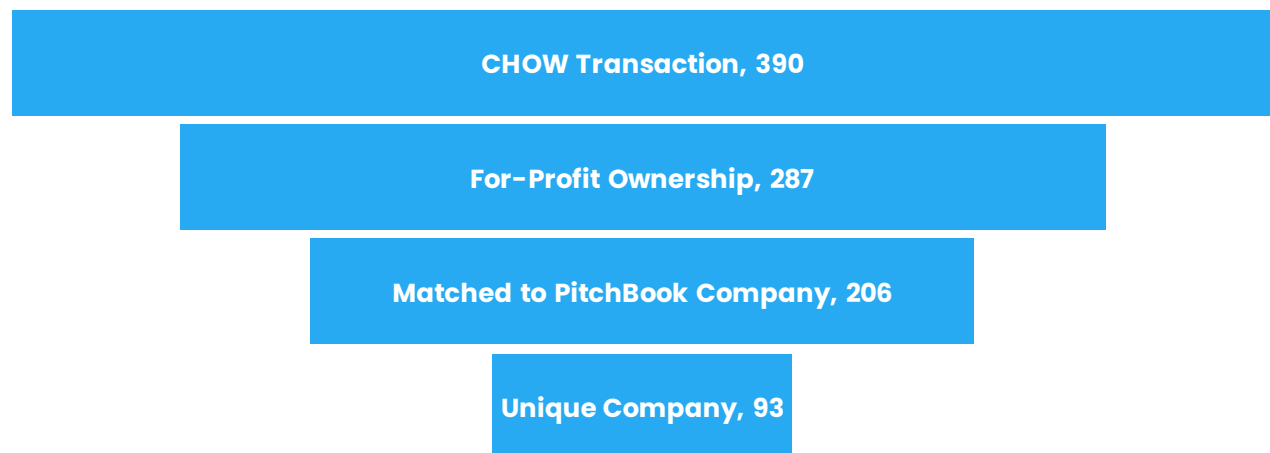
Tracking PE activity across the hospice landscape is not a straightforward endeavor. Private equity ownership is not systematically disclosed in Medicare enrollment records, and many transactions, particularly those involving stock sales or holding company restructuring, occur without generating a public administrative record. This analysis takes a targeted approach by examining the role of PE among hospices that already have a documented signal of ownership activity in the form of a confirmed CMS CHOW transaction.

We started with the complete list of hospices that had a CHOW transaction (390), including hospices that were new entrants and not included in the 2018 landscape. From there, we looked at those hospices that were flagged as for-profit in the Care Compare data (287) and matched 206 hospices in the [PitchBook](#), a Morningstar company, platform.⁸ As CCNs are attributed to individual hospice practices or

⁸ Healthsperien conducted this analysis using accepted best practices. The cited data has not been reviewed by PitchBook analysts and may be inconsistent with PitchBook methodology.

locations, those 206 hospices were attributed to 93 unique companies in the PitchBook database (Figure 4). The remaining 81 hospices were not identified in the database and cannot be included in this deep dive.

Figure 4. Hospices included in the Private Equity Deep Dive, 2025⁹



Role of Private Equity in Hospice Care Over Time

Among the 93 unique companies, which represented 206 hospices, data from PitchBook, a Morningstar company, identified 160 deals occurring on or before December 31, 2025.¹⁰ These deals span private equity, venture capital, corporate and strategic mergers and acquisitions (M&A), and other transaction types. We included deals that occurred before 2018 because the impact of PE ownership, once established, persists in the absence of a subsequent transaction; deals predating the study window can therefore reflect PE relationships that remained active during 2018–2025. Two-thirds of this activity (67.5%, or 108 of 160 deals) nonetheless occurred within the study window, indicating that investment activity among this group of hospices accelerated meaningfully in recent years.

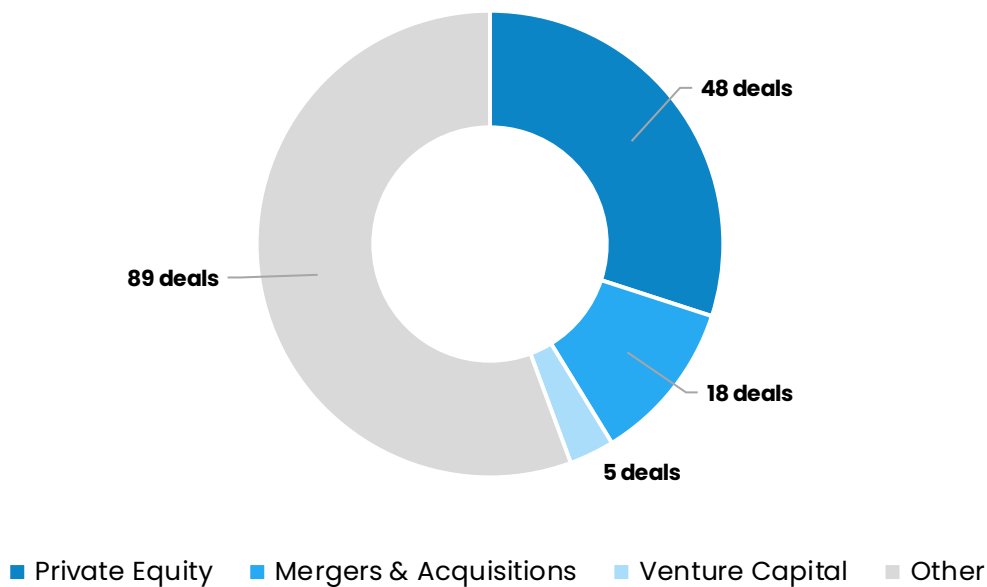
Among the 160 deals that occurred prior to 2026, 30.0% (48 deals) were PE deals. M&A and venture capital deals made up a small portion of the deal landscape, while

⁹ This stepdown count of hospices identified hospices through the CMS Care Compare data as well as company profiles from PitchBook, a Morningstar company.

¹⁰ The overall deal landscape includes an additional 16 deals that were not associated with a specific date. These deals were excluded from the analysis as to not inaccurately attribute the deals to an incorrect date.

other deal types, including debts and refinancing, made up 55.6% (89 deals) of all deals (Figure 5).

Figure 5. Deal Activity, by Type (Prior to 2026)¹¹



While the 48 PE-related deals may seem low compared to anecdotal accounts of PE's expanding role in hospice care, these deals are associated with 111 individual hospices (by CCN). Across the 287 for-profit CHOW hospices, this means that 38.7% (111 hospices) were associated with PE funding at some point through 2025. Because the 81 hospices not found in PitchBook are classified as for-profit, non-PE, this estimate of PE-involvement is likely conservative. Any PE involvement among those 81 hospices would expand the count of for-profit, PE-affiliated hospices beyond 111 of the 287 for-profit hospices.

This analysis of PE involvement in the hospice sector likely understates the true extent of PE's role, given several key data limitations and necessary assumptions. Most notably, there is a lack of comprehensive, publicly available data on hospice ownership that both reflects the current landscape and captures more granular

¹¹ Data from PitchBook, a Morningstar company.

ownership distinctions like PE involvement. Available datasets, such as CMS Care Compare used here, rely on broad ownership categories (e.g., for-profit, nonprofit/government, other) that do not differentiate between types of for-profit ownership, limiting the ability to directly identify PE-backed entities. As a result, we had to link external data sources and proxy methodologies to approximate PE involvement, which may not fully capture all relevant transactions or ownership structures, particularly in cases of complex or indirect investment arrangements.

Discussion

This report highlights how the hospice landscape has changed over time and the limitations of extensive analysis of the landscape using the existing publicly available data on CMS-certified hospices. For-profit hospices now account for a growing majority of hospices across the country, having increased by more than 50% from 2018 to 2025 alone, while nonprofit/government-operated hospices declined. Meanwhile, the role of PE-backed companies continues to make up a substantial presence in the hospice landscape.

Together, these key findings point to increasing consolidation around for-profit hospices, with PE involvement representing a meaningful – and likely undercounted – subset of that growth, even as the nonprofit and government segment of the market continues to contract.

We propose follow-up research should examine how these ownership categories – nonprofit and government, for-profit without identified PE involvement, and for-profit with PE involvement – relate to operational and quality measures, including patient-level outcomes, staffing patterns, and Hospice Care Index performance. The cohorts identified in this report (notably the 111 PE-associated and 176 non-PE for-profit hospices in the deep-dive sample) will form the basis of that comparison.

Taken together, this analysis describes a hospice market in meaningful structural transition—growing rapidly, tilting steadily toward for-profit ownership, and increasingly shaped by private equity activity that current public data only partially captures. While the directional patterns are clear, the precise scope of these shifts, and their consequences for the patients and families who rely on hospice care, cannot be fully assessed with the data presently available. The recommendations that follow identify specific improvements to public reporting that would expand

what researchers, regulators, and the public are able to see, both for routine oversight and for outcomes-focused research.

Policy Implications

Conclusions on the full scope of hospice ownership dynamics, and subsequently how those dynamics impact patients and families who rely on hospice care, remain difficult to assess due to the limitations of publicly available data. These limitations not only constrain academic and policy analysis, but also impede real-time oversight and the ability to identify emerging risks in specific markets or provider segments. Understanding the nuanced relationship between ownership structures and access, quality, outcomes, and costs require accurate and timely data on ownership structures at a minimum.

While CMS has made steps towards increasing transparency in hospice care, including making the Care Compare and CHOW transaction data publicly available, it lacks a mechanism to accurately attribute PE affiliation, holding company structures, or other arrangements that characterize an increasing share of today's market activity. This lack of transparency creates blind spots for regulators and limits the ability to target oversight toward entities whose ownership structures may warrant closer scrutiny.

Updating data reporting requirements to reflect the current breadth of ownership categories, change-of-information activity, parent-company and affiliate-group structures, and private equity affiliations would meaningfully improve the ability of researchers, policymakers, and the public to monitor how the changing hospice market is serving patients and families. Importantly, these updates should be paired with clear definitions, consistent reporting standards, and regular data refresh cycles to ensure usability and accuracy over time. Strengthening ownership transparency in this way is a foundational step toward more targeted program integrity efforts, better-informed payment and policy decisions, and ultimately, greater accountability to the beneficiaries the hospice benefit is intended to serve.

Appendix: Analytic Approach

Methodology

This analysis aimed to refresh prior hospice landscapes and to conduct a deep dive into the increasing role of PE investments. To do so, this study was conducted in two phases: (1) high-level landscape analysis using publicly available data and (2) PE case study using proprietary [PitchBook](#), a Morningstar company data.¹²

To identify hospices that were operating in 2018 and 2025, we used CMS Care Compare data. The Care Compare data provides information on hospice name, ownership type, and location. The Care Compare data is updated quarterly and CMS is required to maintain, and make available, seven years of data. At the start of this study, 2018 was the earliest, complete, year of data available; when all 2025 data files were available, we extended our study to span eight years.

Due to inconsistencies in the quarterly reports, we merged and deduplicated all hospices listed across each of the quarterly 2018 files, based on unique CMS certification numbers (CCN). In order to reflect what the most recent (2025) hospice landscape looks like, we used the final upload of the year; we did not merge files for 2025. We excluded any hospices outside of the US.¹³ We then merged the 2018 and 2025 files by CCN to identify hospices that were operating in 2018 only, 2018 and 2025, and 2025 only. Based on the data provided through the Care Compare Hospice – General Information file, we are able to segment this landscape by geography and self-reported ownership type (for-profit, nonprofit/government, other).¹⁴

After creating the hospice landscape overall, we combined the Care Compare data with CMS Hospice Change of Ownership (CHOW) data to examine changes in

¹² PitchBook, a Morningstar company, is a proprietary, commercial data platform that tracks private capital markets activity, including private equity deals, company ownership, and investment transactions.

¹³ CMS' Care Compare data included hospices in Puerto Rico, Guam, Virgin Islands, and the Northern Mariana Islands that were all excluded from this analysis (46 in 2018, 49 in 2025).

¹⁴ Ownership types are limited to what is available in the Care Compare data, which is based off of self-reported Provider Enrollment, Chain, and Ownership System (PECOS) data. We followed ASPE's methodology to create the nonprofit/government grouping which includes hospices that reported "nonprofit," "government," or "nonprofit/government" as their ownership type. Finally, CMS has no formal definition for the "other" ownership category, which includes ownership arrangements that are less common or unique and allows providers to submit their own values.

ownership. CHOW files are updated quarterly, with information on CHOW transactions, including the effective date, type of CHOW, and the buyer and sellers' CCNs. The 2025 CHOW file included 518 transactions that we then applied several exclusion criteria to in order to align with our study and limit ambiguity in the underlying record. Exclusion criteria included (1) transactions that occurred before our study period (1/1/2018) and outside of the 50 states and DC (reduced transactions to 400); (2) transactions that were missing the sellers' CCN (reduced transactions to 399); and (3) transactions in which the buyer and seller CCNs did not match (reduced transactions to 390). After applying our exclusion criteria, we merged the 390 transactions into our hospice landscape.

The private equity deep dive using PitchBook, a Morningstar company data used hospices that had a reported CHOW transaction as a sample of hospices to examine overall trends that may be occurring across the hospice landscape. To analyze the role of PE investments in hospice care, we started with the complete list of hospices that had a CHOW transaction, including hospices that were new entrants and not included in the 2018 landscape. From there, we looked at those hospices that were flagged as for-profit in the Care Compare data and matched 206 hospices in the PitchBook platform using the hospices' URL to merge individual CCNs with overall companies that would be reported in the platform. As CCNs are attributed to individual hospice practices or locations, those 206 hospices were attributed to 93 unique companies.

Among those 93 companies, we identified all completed deals, of which there were 160 that occurred before 2026. An additional 16 deals that were not associated with any date were excluded from the analysis. Deals were then categorized as (1) PE; (2) venture capital; (3) M&A; and (4) other. This categorization enabled us to compare both the trend of PE funding over time and private interest in the hospice market overall.

Deal types were categorized based off of [definitions provided by PitchBook](#) to include:

- **Private Equity:** Buyout/LBO, PE Growth/Expansion, PIPE
- **Venture Capital:** Seed Round, Early Stage VC, Later Stage VC, Angel (individual)

- **Corporate and Strategic M&A:** Merger/Acquisition, Joint Venture
- **Other:** Any other deal type reported by PitchBook

Finally, we were able to track deals back to individual hospices with a crosswalk that linked individual CCNs to the unique company identification numbers.

Alternative Change in Ownership Approaches

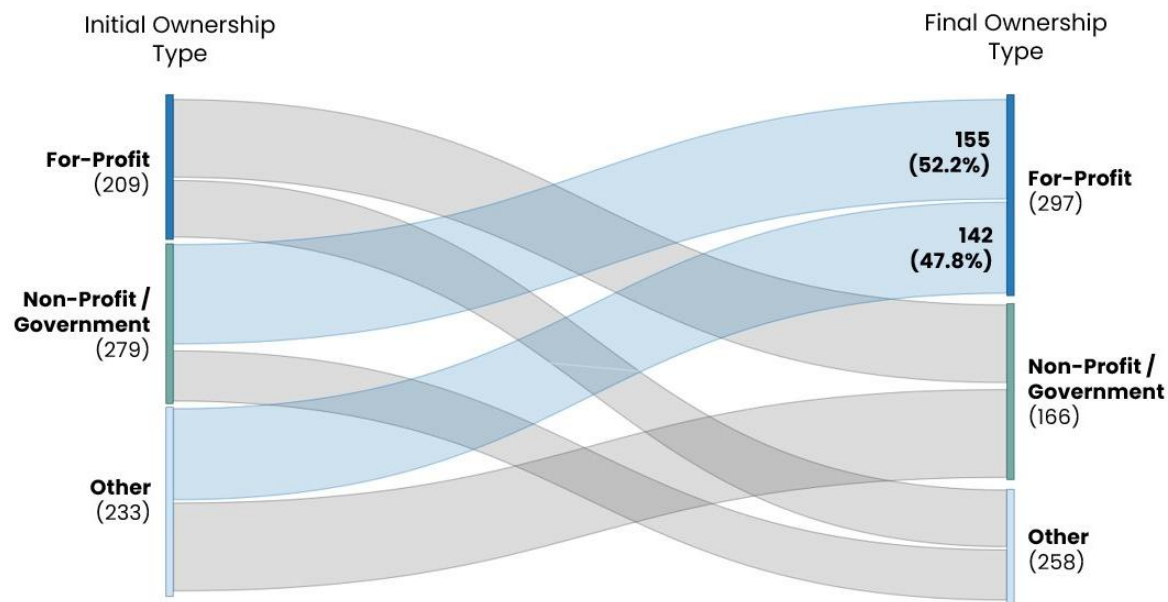
As detailed in this report, we reviewed CHOW transactions to analyze changes in hospice ownership overtime. CHOW transactions, while the only publicly available indicator of hospice ownership change, are limited to a portion of changes.

Transactions that shift operational control without requiring a formal asset transfer, including stock sales, holding company restructuring, and management agreement arrangements, are referred to as a change of information (COI) and do not require CHOW filings. Anecdotally, leaders in the hospice space have opined that COIs make up the majority of changes in ownership, so by limiting an analysis to formal CHOW transactions, there is a possibility of understating the full breadth of activity. To date, COI are not publicly available and cannot be tracked.

To address this potential gap and limitation in the available data, we reviewed the ownership type listed in the Care Compare data among hospices that were operating in both 2018 and 2025. Among the 3,744 hospices operating in both years, we found that nearly one in five (721) hospices reported a different ownership type (tax status) in 2025 than they did in 2018.

The hospices that experienced a change were relatively evenly split across the three ownership categories in 2018, with a slight majority in the nonprofit/government category. Conversely, there was a slight majority of hospices that reported being for-profit by 2025. Along with new for-profit entrants (not shown), this trend reflects the increasing share of the for-profit hospices in the market (Figure A1).

Figure A1. Change in Ownership Type Among Hospices Operating in 2018 and 2025



Changes in ownership as documented by changes to listed tax status in the Care Compare data, however, may also not be reflective of the full breadth of ownership changes in the hospice landscape. For example, if a for-profit owner buys out another for-profit hospice, there would have been a change in ownership that would not be captured using this approach.